

Comprehensive Clinical Brief

James Plunkett, 76 yo male · Updated 2026-05-23 (supersedes 2026-05-04)

Source of truth for treating providers,
U-M ECT intake, ER reference,
family team

Executive summary

James Plunkett is a 76-year-old male with a 45-year history of recurrent major depressive disorder, now in a severe treatment-resistant episode (PHQ-9 = 24, worst in 40 years). The clinical picture has several layers the family is asking the receiving team to assess: (1) severe TRD with documented partial response to IV ketamine in February 2026; (2) a workup pending at U-M Movement Disorders for possible prodromal Lewy-spectrum disease — coded findings include formal RBD (8.5 years), gait abnormality, balance problem, chronic constipation, peripheral neuropathy; DAT-SPECT pending; no confirmed diagnosis at this time; (3) a multi-class activation-intolerance pattern across 2025–2026 trials that the family observes as worth considering in light of the pending Lewy-spectrum question, though attribution is unsettled; (4) a 14-agents-in-12-months psychiatric medication trajectory the family is raising as a concern in light of deprescribing literature, recognizing that high agent count in TRD can also reflect genuine treatment difficulty. **Family and patient have agreed to pursue ECT at U-M (Dr. Maxner)** as the next active step, with start window 2–4 weeks out. The current acute presentation (sleep crisis, motor symptoms, family caregiver burnout) is driving questions about ECT acceleration, bridge medication management, and whether ER/hospitalization is warranted.

Immediate-term decisions facing the family

The questions below are what the family is actively deliberating in the next 24–72 hours. Listed without recommendations attached — listed as the open decisions for which provider input or coordinated family judgment is needed.

1. Sleep regimen — partially resolved as of 5/24 morning. Last night (5/23) Dad took quetiapine 25 mg HS + 25 mg around midnight (total ~50 mg overnight) and slept ~9 hours. No shaking, no orthostatic issues, sedated but not unsteady. **This is the first effective sleep regimen after multi-day deprivation** and confirms the prior tolerance evidence at the 50 mg total exposure level. Daytime question: continue the same regimen tonight (25 mg HS + 25 mg if/when middle-of-night waking) to confirm replicability; this also matches the dose schedule that worked in the 11/2025 trial. **Do not re-introduce doxepin** while nordoxepin still clearing (~through 5/26). **Do not escalate to olanzapine.** Daytime anxious-meltdown management is a separate question — see 5/24 morning update box above + OE #29 differential.

2. Options for voluntary inpatient psychiatric care without going through the ER. OE #27 (5/23) bottom line: the single most leveraged action is **Dr. L's doctor-to-doctor call to Dr. Maxner** Tuesday (5/26) first business day — both for ECT acceleration AND to inquire about direct admission to U-M Geriatric Psychiatry. **Inpatient status accelerates ECT meaningfully:** Kohli 2025 (National Inpatient Sample) median time-to-first-ECT for inpatients = 7.3–9.1 days vs. outpatient queue 2–4 weeks. Direct admission is feasible at U-M but **physician-initiated, not family-initiated** — Dr. L would need to call U-M Psychiatry admissions/on-call attending; holiday weekend staffing is the limiting factor. Voluntary admission well-established in Michigan law and does NOT require meeting involuntary-commitment criteria; severe TRD + sleep crisis + caregiver burnout meets medical-necessity threshold. **Outpatient bridge alternatives (PHP/IOP)** don't provide overnight monitoring, don't accelerate ECT, don't give caregiver respite — limited fit for this presentation. **ER contingency:** if ER becomes unavoidable, walk in with the comprehensive brief printed; lead with "avoid haloperidol and olanzapine — neuroleptic sensitivity profile, Lewy-spectrum workup pending."

3. ECT acceleration via Dr. L-to-Maxner doctor-to-doctor call. The 5/23 plan includes asking Dr. L (covering psychiatrist) to call U-M ECT requesting acceleration of the 2–4-week start window given acute deterioration. *Question:* When can this call happen, and what is the realistic earliest start date U-M can offer?

4. DAT-SPECT scheduling acceleration. The shaking differential (see "The shaking" section below) would be materially clarified by DAT-SPECT. *Question for providers:* Can DAT-SPECT be scheduled within the next 2–3 weeks rather than as part of a longer Movement Disorders queue?

5. Cymbalta dosage through the ECT bridge window. Two short taper attempts (5/12, 5/21 mornings) each produced acute symptoms and reversal. Currently steady-state 60 mg AM. *Question for U-M:* Confirm that holding at 60 mg through ECT initiation (per PRIDE study + Pluijms 2021 meta-analysis on SNRI continuation through ECT) is the appropriate plan, with taper deferred to during or after the ECT course.

Critical alerts for treating providers

- **CONSTELLATION OF FINDINGS UNDER EVALUATION FOR POSSIBLE PRODROMAL LEWY-SPECTRUM DISEASE (workup pending; no confirmed diagnosis).** Findings documented in chart: formal REM Sleep Behavior Disorder (G47.52) 8.5 years; gait abnormality (R26.9 coded 4/14/2026); balance problem (R26.89 coded 7/15/2025); chronic constipation; peripheral neuropathy (G62.9) since 2018. U-M Movement Disorders evaluation pending including DAT-SPECT. **Given this constellation and the categorical avoidance recommendation in Lancet Neurology 2025 for confirmed DLB, the family asks treating providers to consider high-D2-affinity neuroleptics (haloperidol, olanzapine, risperidone) only with explicit consideration of neuroleptic sensitivity risk that would apply if Lewy-spectrum disease is confirmed.**
- **ACTIVATION-INTOLERANCE PATTERN DOCUMENTED ACROSS MULTIPLE TRIALS** in 2025–2026: aripiprazole (parkinsonian features), mirtazapine (agitation), brexpiprazole (balance issues; patient self-discontinued), pramipexole (physical symptoms within days), olanzapine (near-panic-attack rebound on discontinuation), lithium (discontinued by prescriber). The cross-class intolerance pattern is an observation of what has happened; whether it reflects a specific neurobiological substrate (e.g., the Lewy-spectrum question under evaluation) vs. other contributors is a working hypothesis for the U-M workup.
- **HIGH MEDICATION-CHANGE FREQUENCY:** 14+ distinct psychiatric agents in 12 months with brief trial durations and predominantly side-effect-driven discontinuations. The family observes this pattern as a concern in light of deprescribing literature on polypharmacy churn — though whether it is best characterized that way vs. as bad-luck across many trials is itself a question the family wants the receiving team to assess.
- **ECT PLANNED AT U-M (DR. NICHOLAS MAXNER)** — start window 2–4 weeks from 5/21. The family is asking U-M to inherit medication management. Bridge management is the current active question.
- **ACUTE PRESENTATION 5/23:** multi-day sleep crisis (<3 hours/night × several days); new shaking of legs and hands onset 5/17 after doxepin titration to 12 mg; acute anxiety; family caregiver burnout. **Family is considering ER but Dad is declining. Holiday weekend coverage is by Dr. L (covering for Dr. Rubin).**
- **ALLERGY:** Ambien (zolpidem) — reaction "anxiety," Active on EMR.
- **CARDIAC:** Left anterior fascicular block + sinus bradycardia (HR 49–53); BP 112/58; QTcF 401 (5/19/2025 EKG); borderline 1° AV block. Caution with bradycardic / hypotensive / QT-prolonging agents.

5/24 MORNING UPDATE — FIRST EFFECTIVE SLEEP REGIMEN + MIXED DAYTIME PICTURE

Sleep: Dad slept ~9 hours overnight 5/23 → 5/24. Took quetiapine 25 mg HS at bedtime + another 25 mg around midnight (total ~50 mg through the night). First effective sleep regimen after multi-day deprivation.

Morning (per Mom ~9 AM): "Feeling very sedated this morning, but **not unsteady on his feet, no shaking movements**. Going out to Roadhouse now and then a walk." Went out for breakfast.

Mid-morning (per Leah ~10:30 AM): "Dad is melting down. He would clearly prefer that I leave but I'm staying. Someone other than mom needs eyes on this mode." Then ~10:45: "Calmed a bit. Couldn't go through with post-breakfast walk — got too anxious and tired and had to come home. Resting on living room couch. Started to cry then calmed down when I came in at mom's invite to show him a FaceTime with Sam."

Clinical takeaways (carefully calibrated to what we actually observed): (1) **Quetiapine 25 + 25 mg split-dose was effective for sleep with no shaking and no orthostatic/balance issues** — confirms prior tolerance evidence at the 50 mg total exposure level, with sleep indication clearly met. (2) **An anxious meltdown + crying spell + activity intolerance occurred mid-morning**. Mom and Leah described this as anxious distress + tiredness + crying — *not* specifically the inner-restlessness "wants to jump out of his skin" akathisia phenotype. Whether the akathisia phenotype was present this morning is unknown without direct ask of Dad. So we cannot say the akathisia phenotype "persisted despite quetiapine" from this morning's observations alone. What we can say: *if* the meltdown included an akathisia component, then 50 mg quetiapine overnight didn't blunt it; *if* the meltdown was pure anxious distress + fatigue, signals about akathisia treatment response don't apply. (3) **The meltdown calmed with interpersonal contact** (Leah's presence + Sam FaceTime). This is informative for the differential *if* any akathisia component was present (akathisia typically doesn't respond to interpersonal soothing while anxious depression often does), but is also straightforwardly consistent with pure anxious distress responding to family support. (4) **Activity intolerance pattern** — couldn't complete post-breakfast walk; got too anxious + tired and returned home. Consistent with severe depression baseline + cumulative-sleep-deficit recovery + possible post-prandial autonomic component. See OE #30 for systematic interpretation.

Practical implication for the next 24–48 hours: if the family + Dr. L are comfortable, continue the 25 mg HS + 25 mg midnight split-dose pattern for one more night to confirm replicability. Daytime anxious-meltdown management is a separate question and is part of the broader case for accelerating ECT / inpatient (handout #28).

Current acute presentation (2026-05-23 baseline; 2026-05-24 morning update above)

Domain	State
Sleep	<3 hours/night for several days. Lemborexant tried 5/22 night — 10 mg tablets prescribed; taken as half-tablet + half-tablet ≈ 10 mg total (maximum labeled dose); ineffective. Doxepin + trazodone discontinued ~5/21. No effective sleep regimen currently.
Motor	Shaking of legs and hands. First acutely noted 5/17 morning, after doxepin titration to 12 mg HS the prior night (no other med change). Persistent at some level since 5/17. Worsening with sleep deprivation. Whether shaking predates 5/17 in milder form is uncertain (Mom unable to confirm). 5/23 AM episode required Mom to physically hold Dad. Distribution and pattern per Mom (5/23 characterization): legs and hands both; "like shaking when you're cold"; does NOT occur during walking; "doesn't last long" — episodic and brief rather than continuous. Pattern more consistent with episodic rest/enhanced-physiologic tremor than with continuous myoclonus or action tremor.
Affective	Acute anxiety; intermittent reported despair. Suicidal ideation has not been reported in recent assessment. Last documented PHQ-9 = 24, GAD-7 not recently retested.

Domain	State
Cognitive	Documented "brain fog" (R41.89). Spotty short-term memory in recent calls; less impaired than San Francisco period earlier. Pending U-M Cognitive Disorders Program evaluation.
Autonomic	One nocturia trip per night (down from prior pattern). Eats with appetite support; doesn't register hunger spontaneously. Continued unintentional weight loss (R63.4 coded 4/14/2026). Constipation managed with Metamucil + MiraLax.
Caregiver state	Wife Marcy primary in-home caregiver — exhausted, sleep-deprived, expressing "can't go through this again." Adult children debating early weekend travel for support. Sister-in-law Debbi arrives 5/26.

Current medications (2026-05-23)

Medication	Dose/timing	Status / notes
Duloxetine (Cymbalta)	60 mg AM before breakfast (~8 AM)	Active. Steady state since 11/2025. Two 1-day taper attempts at 40 mg (5/12 and 5/21 mornings), each reversed within 24 hours. Recommendation: hold at 60 mg through ECT per published evidence (PRIDE study + Pluijms 2021 meta-analysis support SNRI continuation through ECT).
Lorazepam (Ativan)	1 mg PRN	Active PRN. Most recent doses: 1 mg on 5/22 PM and 1 mg on 5/23 ~8 AM. April–May dosage history: First dispensed 0.5 mg 9/9/2025; patient self-reported low PRN use (~4 doses total) through 4/21/2026. Dose increased to 1 mg by 4/2/2026. Per Rubin's 4/23/2026 plan: 1 mg daily at midday as time-limited wash-out bridge — daily use confirmed by family observation by 5/4/2026. Subsequently tapered (per Rubin + Peltzman plan) and reverted to PRN-only use. Family concern: trajectory back toward daily use given current acute crisis.
Melatonin	10 mg HS	Active. Prescribed for RBD since 10/31/2017 — same day as G47.52 diagnosis (not for generic insomnia).
Lemborexant (Dayvigo)	10 mg tablets prescribed (filled 5/22 CVS, \$335)	First trial 5/22 night: Dad took half a tablet (5 mg) initially, then another half (5 mg) later — total ~10 mg (maximum labeled dose). Ineffective for sleep. The next-night question is whether a single 5 mg or 10 mg dose taken once at the start of the night (rather than split) performs differently, and whether sleep failure tracks the medication or the acute sleep-deprivation/anxiety substrate.
Tadalafil	5 mg daily / 20 mg PRN	BPH / ED. No psychiatric interactions of concern.
Rosuvastatin	20 mg	On PCP active list; recent CVS fill pattern uncertain.
Metamucil / MiraLax	per label	Constipation. OTC.
B12 supplementation	per regimen	For methylmalonic acidemia (MMA 0.46 H, B12 408).

Recently stopped (past ~4 weeks)

Medication	Stopped	Reason
Doxepin (Silenor)	~5/21	Dosage history: Started 6 mg HS ~5/4/2026 (per Rubin; family report; CVS stock was an issue on 5/4 so actual first dose may have been a day or two later). 6 mg HS maintained for ~12 nights through 5/15. Titrated to 12 mg HS for one night only — 5/16 (per family decision in consultation with Rubin). After first acute shaking onset 5/17 morning, Dr. Rubin reduced back to 6 mg HS (per handout #18 framing — "Dr. R's first move was taking doxepin back to 6 mg"). 6 mg HS continued ~5/17 through ~5/20 (~4 nights). Discontinued ~5/21 as part of sleep-stack transition to lemborexant. Note: the single-night 5/16 titration to 12 mg HS coincided with first acute shaking onset 5/17 morning — only med change that night. Nordoxepin (active metabolite) has 31-hour half-life and accumulates over consecutive nights — residual nordoxepin from cumulative dosing likely still meaningfully on board for several days post-discontinuation (~through 5/26).
Trazodone	~5/21	Discontinued as part of sleep-stack transition to lemborexant.
Olanzapine (Zyprexa)	4/21/2026	Discontinued by Dr. Rubin via phone call. ~12 days on board (~6 days at 5 mg/day). Near-panic-attack rebound on first full day off.
Lithium	4/21/2026	Discontinued by Dr. Rubin same call. ~18 days on board at 150 mg. Tremors prior to discontinuation; resolved post-discontinuation.
Pramipexole	~3/26–28	Discontinued by Dr. Rubin within days of start due to physical symptoms.
Brexpiprazole (Rexulti)	~mid-March 2026	Self-discontinued — patient-stated reason: balance issues. ~3 months on board (CVS fills 12/1/2025, 1/5/2026, 1/30/2026, 3/2/2026).
Divalproex (Depakote)	~5/13/2026	Brief trial — was actually filled and started, not just "proposed." CVS fill 5/10/2026 (125 mg DR tabs). Started shortly after. Discontinued ~5/13 per Adam's 5/14 note ("dad stopped depakote yesterday"). Duration ~3 days. Indication was acute anxiety augmentation per Rubin (Chilton noted but unclear if recommended).

Proposed / under discussion (not yet on board)

Medication	Proposed by / when	Status / family read
Quetiapine (Seroquel)	Dr. L (covering psychiatrist) — 5/23. Also previously filled 5/17/2026 (25 mg tabs, per pharmacy CSV) in response to the 5/17 shaking episode. Actually used 5/23 night: 25 mg HS + 25 mg around midnight (~50 mg total overnight) → ~9 hours sleep; no shaking; no orthostatic issues.	Now actively in use for the sleep bridge. Tier 2–3 option (revised from Tier 4) per OE #28 follow-up. Prior tolerance evidence: the 11/2025 trial (50 mg × ~4 weeks) was discontinued because it "wasn't doing anything but sedating" — an efficacy failure at an anxiety indication, NOT a safety or activation-intolerance failure. Sedation IS the desired effect for the sleep indication. The cardiac substrate (LAFB, HR 50, PR 214, QRS 120, QTc 401 on 5/19/2025 EKG) was the same during the 11/2025 trial; the patient cleared it with no documented adverse event. The PCP has not refreshed the EKG or escalated the cardiac findings in 12+ months, suggesting these findings are stable and not blocking standard psychiatric prescribing. Initial OE #26 chronic-use safety signals (Hui 2025 HR 3.1 mortality / HR 8.1 dementia / HR 2.8 falls; Højlund 2022 aHR 1.52 MACE) are months-of-exposure endpoints that don't apply meaningfully to a 2–3 night bridge; the HR 8.1 also almost certainly overestimates causal effect due to residual confounding-by-indication (elderly insomnia patients prescribed quetiapine are enriched for pre-existing cognitive complaints that are themselves early prodromal-dementia markers). For tonight specifically: if pursued, start at 25 mg HS (not 50 mg) given (a) the residual nordoxepin (active through ~5/26; wasn't present in 11/2025 trial), (b) lowest-effective-dose principle in possible prodromal Lewy-spectrum disease, (c) the option to escalate to 50 mg on Night 2 given prior tolerance. Fresh ECG is ideal but not an absolute barrier given prior tolerance at the same dose with the same cardiac substrate. The "Tier 4 / recommended against" framing from initial OE #26 was over-calibrated for this patient's specific case.
Olanzapine (Zyprexa)	Dr. L — 5/23 (second-line if Seroquel fails)	Family is concerned. Already discontinued 4/21 for activation-intolerance pattern; higher D2 affinity than quetiapine; Lancet Neurology 2025 categorical avoidance recommendation in confirmed DLB-spectrum patients (Dad's Lewy-spectrum workup is pending). Family asks: should not proceed without explicit U-M input.
Cymbalta → Lexapro switch	Drs. Rubin + Chilton — 5/19 (pre-Royal Oak crisis)	Stated rationale: reduce pre-ketamine anxiety. Escitalopram 10 mg was actually filled at CVS on 5/11/2026 (pharmacy record), but family ended up deferring on starting it per Adam's 5/14 reasoning. Family concern: SNRI can be continued through ECT per published evidence (PRIDE study, Pluijms 2021), and SSRI initiation does not reliably reduce anxiety in 2–4 weeks (often increases it; jitteriness syndrome 20–30% in elderly).

Critical clinical timeline

Long arc (decades pre-2020 → 2024)

Decades pre-2020	Long-term escitalopram (Lexapro). Steady-state up to 30 mg for many years — above standard maximum. Brief 40–50 mg trials in 2023 felt worse ("uncomfortable, agitated, more anxious" per Dad's 1/26/2024 letter to NP). Activation-intolerance signature is documented from 2023 — not a 2025 emergence.
2016-07-22	MDD single episode (F32.9) first added to PCP problem list.
10/31/2017	Formal REM Sleep Behavior Disorder (G47.52) diagnosed. Melatonin 3 mg started same day for RBD specifically (not for generic insomnia).

11/26/2018	Peripheral neuropathy (G62.9) added to PCP problem list.
2020 summer	Per Adam: "as good a time as I can remember having in his adult life." Baseline-good functional state.
2020-08-10	PCP encounter with Dr. Oberdoerster; F32.9 among encounter diagnoses.
2020 November	Statin switch: simvastatin 40 mg → atorvastatin 20 mg (per prior PCP for LDL control).
2020 December	The "cliff." Rapid onset of sustained anxiety + depression unlike anything Dad had previously experienced. Patient-and-Adam working hypothesis: the atorvastatin switch pushed him past his SSRI-tolerance ceiling. Hypothesis never cleanly retested; rosuvastatin switch eventually requested in 2024.
2021 fall	Too depressed to focus on reading (had previously given Adam thoughtful book notes).
2022 fall	Worst visible state at that point — "worse than 2022 is hard to imagine" became Adam's frame of reference.
2023-11	Walk with Adam + Mom: "the world was weighing on him."
2023-12-05	PCP annual wellness with Dr. Straka. Full neurologic exam normal, nonfocal . Motor strength normal upper and lower; sensory intact. No gait findings. Weight 183 lbs; HR 53; BP 128/82.
2023 December	Berit (NP): "Occam's razor is he's just depressed." Considered mirtazapine (not started then).
2024-01	Adam helped draft letter to NP requesting statin trial switch (atorvastatin → rosuvastatin). Followup unclear.
2024 February	Rosuvastatin start 2/1/2024 per EMR (atorvastatin actually continued on Birdi mail-order fills through 6/10/2024 — actual switch mid-to-late 2024).
2024-02-28	Frequency of micturition (R35.0) coded on problem list.
2024-03	Raw-nerve evening; beer + ~most of a bottle of wine. Adam pushes about drinking and about seeing psychiatrist. Dad: "I don't have dementia yet."
2024-04	Stopped drinking. Adam: "Best I've seen him since early March 2021." A natural-experiment data point: alcohol is a real contributor, not a red herring.
2024-06-14	Sleep disorder (G47.9) coded on problem list.
2024-12-10	PCP annual wellness. "Starting TMS treatment in addition to Lexapro." Mood disorder (F39) coded. Weight 176 lbs (down 7 lbs from 12/2023). HR 49 (bradycardia); BP 112/58. Advance care planning addressed.
12/2024	Standard TMS course begins. Course outcome / response / discontinuation reason not in family records (pending TMS provider chart).

2025 — escalating treatment resistance + first cognitive observations

04/02/2025	Aripiprazole 2 mg augmentation trial begins (CVS fill).
2025-04	Leah (Adam's sister) notices short-term memory and general-functioning lapses during Seattle visit — flags to Adam. <i>This establishes that some cognitive slippage predates the current acute episode by ≥1 year.</i>
2025-05	Disc-golf / charity-discs story (5/29–30): warm, engaged, elaborate recall of conversations. Real improvement.
05/19/2025	Recurrent depression (F33.9) coded. EKG: LAFB, PR 214 (borderline 1° AV block), QRS 120, QTc 401, HR 50. PCP note: "Doing better on Abilify, QT shorter than in 2017."
07/15/2025	Balance problem (R26.89) coded. CT head: stable. First IHA Neurology referral. PCP note: gait "essentially normal though somewhat hesitant... some word finding problems but otherwise no focal deficit."
07/17/2025	Finasteride 5 mg started (BPH). Single fill. Labeled depression/SI warning.
2025-07	Discovery Park visit. Cheerier, more engaged, less reactive. Converting to Judaism with rabbi — substantive meaning-making practice.
07–10/2025	Mirtazapine trial (30 → 45 → 30 mg). Stopped for agitation . Activation phenotype.

2025 autumn Per Dec 2025 ChatGPT thread: **Lexapro** had "grown ineffective"; **Seroquel** "wasn't doing anything but sedating" → **switched**; **Remeron** "more agitated"; **Rexulti** "more anxious." Activation-intolerance signature continues across multiple agents.

09/09/2025 Constipation (K59.09) coded. Lorazepam 0.5 mg first dispensed. **PHQ-9 = 21 (Severe)**; **GAD-7 = 16 (Severe)**. **SI "Not at all."**

10/05/2025 Escitalopram course ends after ~decades of long-running use.

09/30/2025 Duloxetine started 20 mg. Titrated to 60 mg by 11/03/2025.

11/03/2025 Quetiapine 50 mg started — first fill. Used ~4 weeks. Discontinued because "wasn't doing anything but sedating" (efficacy failure at anxiety indication, not safety).

11/04/2025 Drug-drug interaction (Z78.9) coded — "please call your psychiatrist." Specific agents not in available records.

12/01/2025 Brexpiprazole (Rexulti) started 0.5 mg → 1 mg by 1/5/2026. ~3 months on board (CVS fills 12/1/2025, 1/5/2026, 1/30/2026, 3/2/2026).

12/16/2025 Psychophysiological insomnia (F51.04) coded. PCP counseled against Benadryl ("fall and possible dementia risk"); suggested asking psychiatrist about trazodone. **PHQ-9 = 17 (Moderately Severe)**; **GAD-7 = 7 (Mild)**; **SI "Not at all."**

12/17/2025 Trazodone 50 mg single fill — tension/headache; quarter-pill PRN remained available.

12/22/2025 Sleep workup (via Adam). Sleep stack: trazodone 50 mg + melatonin 3 mg + Rexulti 0.5 mg HS + Cymbalta 60 mg AM. **3–4 nocturia episodes/night**. Late afternoon caffeine flagged. Nocturia identified as likely the single biggest sleep disruptor.

February–April 2026 — ketamine response, procedure, decompensation

01/2026 Ketamine decision made (at the clinic where TMS had been done).

02/2026 Marin County ketamine course (sublingual lozenge, 10–20 mg per session, ~4 weeks). **"Aged in reverse by ten years"** per Adam. Anxiety lifted; sustained engagement; sense of wonder; disc golf. ~4 weeks of this state. Cognition, mood, physical activity all materially improved.

2026 early March Visit (3/7/2026 entry): meaningfully better. "Consistently more engaged and very responsive to Maren." Mixed signal: did not close the bathroom door while urinating at Adam's house — possible subtle executive-cognition lapse (not by itself diagnostic; worth noting).

2026 early March Depression beginning to return (retrospective self-report). Ketamine paused ahead of procedure.

2026 mid-March Patient self-discontinued brexpiprazole — stated reason: balance issues.

03/18/2026 Aquablation procedure (water/robotic) under general anesthesia. Technically successful. Recovery slow; sleep still disrupted from prostate inflammation; could not resume workouts for weeks.

03/24/2026 Pramipexole 0.125 mg dispensed; discontinued within days for physical symptoms (per Dr. Rubin).

04/03/2026 Lithium 150 mg started.

04/06–12/2026 April 12 call: Dad candid. New supplement briefly added then dropped; "hadn't tried lithium before." Weight loss substantial; Mom reversed course and is pushing fat + protein. Ketamine paused; wants to stabilize first. Not feeling hungry the way he used to.

04/08/2026 Family-noted physical symptoms.

04/09/2026 Olanzapine 2.5 mg started; increased to 5 mg PM of 4/14 after continued anxiety report.

04/14/2026 PCP visit. Gait "essentially normal though somewhat robotic." R26.9 (gait abnormality) + R63.0 (decreased appetite) + R63.4 (unintentional weight loss) all newly coded. New IHA Neurology referral ("Rule out Parkinson's"). PCP recommended ECT consideration. Labs drawn (CMP, CBC, TSH, B12, A1c — all WNL except BUN 21 H, ALP 39 L).

04/15/2026 Dr. Rubin orders lithium level + BUN/Cr + TSH + calcium. Reported normal verbally 4/21; numerical values not yet in family hands.

- 04/17–19/2026** **Wedding weekend nadir.** Cognitive lapses ("forgot basic plans, a close friend's job, the prior day's emotional conversations"). Halting + slightly stooped gait; hypophonia day 1; balance unsteadiness; possible tremor reported by others; vacant look with resting open mouth at times; reduced facial expressiveness. Per Dad: almost didn't come due to social anxiety. Sign cluster has overlap with Bush-Francis catatonia screen.
- 04/21/2026 PM** Dr. Rubin phone call: discontinue lithium AND olanzapine.
- 04/22/2026** First full day off both. Reported near-panic-attack rebound — closest to panic in decades per Dad.
- 04/23/2026** Resolved plan with Rubin: maintain duloxetine 60 mg; restart ketamine with maintenance from day one; ECT held as contingent backup; lorazepam as time-limited bridge (initially PRN, escalated to 1 mg daily midday by 5/4); trazodone 50 mg fresh fill at CVS this day.
- 04/27/2026** Dad on bike after Ativan; Dad shows up for family vacation; takes another Ativan dose. Adam observes; family pushback active.

Recent acute course (May 2026)

- 05/04/2026** Updates: Lorazepam now 1 mg daily midday as bridge per Drs. Rubin + Peltzman. ZZZQuil being used recently — family agreed to stop. Alfuzosin confirmed discontinued post-prostate-procedure. PCP visit: testosterone panel returned — free T 3.6 (L), bioavailable T 82 (L), SHBG 70.1 (high-normal) — biochemical hypogonadism. Doxepin 6 mg HS proposed and started around this date.
- 05/05/2026** Ann Arbor ketamine clinic restart originally targeted (later pushed to 5/18).
- 05/10/2026** Divalproex (Depakote) 125 mg DR filled at CVS (per pharmacy CSV). Started shortly after for anxiety augmentation per Rubin proposal.
- 05/11/2026** Escitalopram (Lexapro) 10 mg filled at CVS for the proposed Cymbalta→Lexapro switch. Duloxetine 20 mg cap also filled (used for the 5/12 taper attempt as $2 \times 20 = 40$ mg). Family ultimately deferred on starting Lexapro per Adam's 5/14 reasoning.
- 05/12/2026** First Cymbalta taper attempt — 40 mg morning dose (2×20 mg caps). **1 day only.**
- 05/13/2026** Returned to 60 mg morning after psychiatrist consult. Family discussed postponing further tapering until after ketamine.
- 05/13/2026** **Depakote discontinued** (per Adam's 5/14 note: "dad stopped depakote yesterday"). Brief ~3-day trial.
- 05/14/2026** Julane (ketamine integration therapist) shares preferred sequencing — would prefer running first ketamine sessions on existing stack before any Cymbalta changes. Dad reports RBD episodes now once every ~2 months, less frequent than 2017–18.
- 05/15/2026** Sleep aid discussion. Dad asks about Dayvigo and other options.
- 05/16/2026** Plan to titrate doxepin to 12 mg; consider Remeron as alternative. Productive call with Dr. Rubin; Rubin willing to coordinate with Dr. Chilton; willing to give Dad options. **Doxepin titrated to 12 mg HS this night.**
- 05/17/2026** **First acute body movements / shaking noted** — morning after doxepin 12 mg first dose. Other meds unchanged. **Dr. Rubin reduced doxepin back to 6 mg HS** as first move (12 mg was a one-night dose only). Quetiapine 25 mg tabs also filled at CVS same day (per pharmacy CSV) — connects to OE prompt #08 proposed regimen for the shaking; status of whether actually taken uncertain.
- 05/18/2026** Royal Oak ketamine session. Difficult/dissociative — "convulsive" per Mom; Dad described as "going to Africa" / runaway train. Shaking present during session. Post-session: more coherent than Mom had seen him in ages, but Dad did not feel better. Mom used word "retraumatizing."
- 05/19/2026** Crash continues. Calls with Julane. Drs. Chilton + Rubin propose Cymbalta → Lexapro switch ahead of next ketamine session; depakote mentioned by Chilton (unclear if recommended).
- 05/21/2026 AM** Second Cymbalta taper attempt — 40 mg morning dose (per Mom's 5/23 consolidated recall; contemporaneous documentation places dose earlier the night before). Acute anxiety crisis through 5/21 day–night; lorazepam 1 mg used to stabilize. Covering provider (Rubin proxy) directed return to 60 mg.

05/21/2026	Family + Dad agree on ECT at U-M as the active path. Mom meets with Dr. Maxwell, Director of ECT, U-M. Callback from Dr. Maxner requested. ECT start window 2–4 weeks. Doxepin and trazodone discontinued. Lemborexant 5 mg HS planned to start.
05/22/2026 AM	Returned to 60 mg Cymbalta. Sleep regimen transition continues.
05/22/2026 PM	Shaking episode.
05/22/2026 night	Lemborexant tried for first time — 10 mg tablets prescribed; taken as half-tablet (5 mg) + half-tablet (5 mg) later in the night, total ~10 mg (max labeled dose). Did not produce sleep.
05/23/2026 AM	Intense shaking of legs and feet — Mom physically holding Dad. Ativan 1 mg taken at ~8 AM. Mom calls Sharon (family psychiatrist friend) and drafts script for Dr. L (covering for Dr. Rubin). Plan proposed: Seroquel 25 mg HS as sole sleep therapy; possible escalation to Zyprexa if ineffective. Doctor-to-doctor call to U-M ECT requested to accelerate. Family debating ER.
05/23/2026 night	First effective sleep regimen. Quetiapine 25 mg HS at bedtime + 25 mg around midnight (total ~50 mg overnight). ~9 hours sleep.
05/24/2026 AM	Sedated but stable; no shaking, no unsteadiness on feet. Went out for breakfast at Roadhouse with Mom.
05/24/2026 ~10:30 AM	Post-breakfast meltdown. Too anxious + tired to complete post-breakfast walk; returned home. Crying spell on living-room couch; calmed when Leah came in and Mom invited Sam FaceTime. Leah: "Someone other than mom needs eyes on this mode."

The shaking — what we know so far

First acutely noted: 5/17/2026 morning. The only medication change the prior night (5/16) was doxepin titration from 6 mg to 12 mg HS. No other psychiatric medication change.

Persistence since 5/17: "Some of this shaking evident ever since then" per Mom (5/23 clarification).

Whether shaking predates 5/17: Uncertain. Mom: "As to whether it was there earlier (in addition to general anxiety and ruminations), I don't know." Possible mild background not previously noted.

Acute exacerbations: 5/18 Royal Oak ketamine session; 5/22 PM; 5/23 AM. Tracks with sleep deprivation, acute anxiety, and Cymbalta taper-and-reversal events.

Distribution and pattern (Mom characterization 5/23): Affects legs *and* hands. "Like shaking when you're cold." Does **not** occur during walking. "Doesn't last long" — episodic and brief rather than continuous. Pattern more consistent with episodic rest / enhanced-physiologic tremor or drug-induced tremor than with continuous myoclonus or action tremor.

Differential — in rough order of weight (revised given 5/23 Mom characterization)

Mom's 5/23 description (legs and hands; "like shaking when cold"; not during walking; brief / episodic) shifts the differential toward tremor-pattern phenomena (rest, postural, enhanced-physiologic, or drug-induced) rather than continuous myoclonus or PLMD specifically.

- Doxepin / nordoxepin-related tremor** — temporally most proximate. The 5/16 single-night titration to 12 mg HS (a dose that moves from pure-H1 territory toward anticholinergic and 5-HT_{2A} effects) coincided exactly with the 5/17 shaking onset. Dr. R reduced back to 6 mg the next day, but cumulative nordoxepin (active metabolite, 31-hr half-life) accumulates over the full ~17-night doxepin course and persists ~5 days post-discontinuation. Doxepin fully discontinued ~5/21; residual nordoxepin likely meaningfully present through ~5/26.
- Prodromal Lewy-spectrum rest tremor** — Dad's underlying vulnerability (formal RBD 8.5 years, gait abnormality, balance findings) is consistent with early Parkinsonian rest tremor. Rest tremor (intermittent, both hands and legs, not during ambulation) fits Mom's description. The doxepin titration may have unmasked or worsened an underlying signal.
- Enhanced physiologic tremor** — the "like shaking when cold" descriptor matches enhanced physiologic tremor (which is what cold shivering essentially is). Drivers: extreme sleep deprivation, acute anxiety, caffeine, recent stimulant exposure, hypoglycemia. All relevant in current context.

- 4. **Residual drug-induced tremor** — from recent olanzapine, brexpiprazole, lithium, pramipexole exposures (March–April). Lithium-induced tremor specifically can persist weeks-months post-discontinuation. Tremors were noted on lithium prior to 4/21 discontinuation and reported as "resolved" — but underlying signal may have re-emerged.
- 5. **SNRI-related tremor** — SSRIs/SNRIs are documented to produce tremor at steady state. The 5/12 and 5/21 taper attempts may have produced acute withdrawal-related tremor contributions on top of any steady-state baseline.
- 6. **Acute anxiety tremor** — episodic brief tremor in legs and hands during acute anxiety episodes is consistent with Mom's description. May co-occur with the above.
- 7. **RBD exacerbation / dream-enactment** — flagged in family quetiapine handout (5/17) as a possibility, since (a) antidepressants including duloxetine can trigger/worsen RBD, (b) Dad has formal RBD (G47.52) 8.5 years, (c) the 5/17 onset was morning-after-sleep. Less consistent with Mom's 5/23 characterization (episodic, brief, "like shaking when cold," not associated with dream recall per Dad's recent reports). Worth asking Dad whether he recalls dream content with morning episodes.

Less consistent with Mom's description: continuous myoclonus, PLMD (periodic limb movement disorder, which occurs in sleep), or action tremor. The "not during walking" detail is especially diagnostic.

Family's contemporaneous analytical view (per handout #18, 5/17): The most parsimonious initial intervention is reducing doxepin back to 6 mg HS rather than layering on a new agent. The handout #08 framing also applies: don't run two pharmacological changes simultaneously in a 76-year-old when one of them addresses the most plausible cause directly. Doxepin has since been discontinued entirely (~5/21), so this question is partially resolved — but residual nordoxepin remains relevant through ~5/26.

Cymbalta taper history — two attempts

Date	Dose	Outcome
5/12/2026 AM	40 mg (from 60)	One-dose-only attempt. Returned to 60 mg the next morning after psychiatrist consult. Family discussed postponing further tapering until after ketamine.
5/21/2026 AM	40 mg (from 60)	Second one-dose attempt per Rubin's prescribed schedule. Acute anxiety overnight 5/21→5/22 AM; lorazepam 1 mg used to stabilize; covering provider directed return to 60 mg.

Net status: 60 mg morning dosing maintained except for two single-dose attempts at 40 mg, each reversed within 24 hours. Currently steady-state at 60 mg AM since 5/22.

Go-forward plan informed by family concern (consistent with PRIDE study + Pluijms 2021 meta-analysis): hold at 60 mg through ECT initiation; let U-M manage taper during or after ECT response when patient is under closer monitoring and ECT itself is providing antidepressant effect.

Load-bearing clinical uncertainties

1. What is causing the shaking?

Differential above. The single highest-leverage diagnostic question. **Implications:** if proximate cause is doxepin/nordoxepin, time off doxepin should resolve over ~5 days post-discontinuation; if it reflects unmasking by the underlying findings under Lewy-spectrum workup, the question becomes U-M Movement Disorders'; if it's sleep deprivation, resolving sleep is the treatment.

2. Whether Seroquel is the right bridge agent

Quetiapine has the lowest D2 affinity of atypicals and is the only antipsychotic generally considered usable in confirmed DLB. **But:** Dad already failed a single trial 11/2025 with reason undocumented; his activation-intolerance pattern includes multiple antipsychotics; the proposed escalation to Zyprexa if Seroquel fails is the wrong direction given his profile. If Seroquel proceeds, dose entry at 12.5 mg and explicit motor-symptom monitoring are warranted.

3. ER decision — holiday weekend risk-benefit

Arguments to go: acute sleep crisis can escalate; parenteral options for monitored stabilization; psychiatric admission could accelerate ECT; Mom is exhausted. **Arguments against:** ER physicians may default to haloperidol or olanzapine (categorically contraindicated in Dad's profile); Dad's complex history hard to communicate quickly; involuntary admission criteria not currently met. **If ER is pursued, advance briefing of ER physician on neuroleptic-sensitivity profile is critical.**

4. ECT acceleration timing

U-M ECT start window 2–4 weeks from 5/21. Doctor-to-doctor call from covering psychiatrist citing acute deterioration is reasonable and was on the 5/23 plan. Whether U-M can accommodate sooner-than-3-weeks is the practical question.

5. Lorazepam trajectory

1 mg used 5/22 PM and 5/23 AM. Daily-use trajectory is the concern. Dad has prior completed lorazepam taper (was on 1 mg daily midday confirmed by 5/4, after a ~late-April Rubin-prescribed wash-out bridge plan). Re-induction may be faster than first-time use. Family concern: avoid scheduled use; U-M to inherit the lorazepam management question post-ECT.

6. Sleep regimen for the bridge window

Dayvigo at full 5 mg dose hasn't been fairly tested (5/22 trial was half + half = sub-therapeutic). Options to consider: full-dose Dayvigo 5–10 mg HS; return to doxepin + trazodone if pre-shaking sleep was acceptable; clonazepam 0.25–0.5 mg HS (single-mechanism: addresses RBD + sleep + anxiety, but introduces another benzodiazepine); gabapentin or pregabalin (non-neuroleptic, can address anxiety + sleep + possible RLS).

7. The Cymbalta-vs-Lexapro question medium-term

Chilton/Rubin's stated rationale (reduce pre-ketamine anxiety) is at odds with the actual pharmacology of SSRI initiation in weeks 2–4 (often increases anxiety; jitteriness syndrome incidence 20–30% in elderly). PRIDE study + Pluijms 2021 support continuing SNRI through ECT. Most parsimonious plan: hold at 60 mg through ECT; let U-M decide post-response.

8. RBD framing: idiopathic vs. SSRI-unmasked

Dad's formal RBD diagnosis (G47.52) is 8.5 years old (10/2017), and developed during long-term escitalopram use (decades pre-2020 on Lexapro 30 mg). **Iranzo's foundational iRBD cohort explicitly excluded patients on SSRIs** — so the standard "iRBD → 70–90% conversion to synucleinopathy" figure may not apply directly. **Postuma 2013 (N=100): SSRI-associated RBD converted at 17% over ~4.5 years vs 52% for iRBD (RR 0.22, 95% CI 0.06–0.74).** Prodromal biomarkers (olfaction, constipation, MCI) look similar across both groups, suggesting SSRIs may unmask the phenotype rather than cause it — but the conversion rate is meaningfully lower in SSRI-context cases. Implication: the Lewy-spectrum prior may be lower than the iRBD literature suggests. Parasomnia overlap disorder and OSA-pseudo-RBD remain in the differential. U-M Movement Disorders workup + DAT-SPECT will refine.

9. The cognitive trajectory predates the current acute episode

Leah noticed short-term memory and general-functioning lapses during her April 2025 visit (over a year ago) — well before the current acute decompensation. The appetite-interoception dissociation is 1–2 years old per family report. The "I don't have dementia yet" comment was March 2024. **Something cognitive has been moving gradually underneath the psychiatric sawtooth for years** — not all of it explainable by the recent acute episode. This is what the U-M Cognitive Disorders Program evaluation is positioned to characterize.

10. Prognostically important — Dad's brain has come back online multiple times

"Every 'up' has been real but short" across the past several years: April 2024 (alcohol cessation) → "best I've seen him since early March 2021"; May 2025 (disc-golf weekend); July 2025 (Discovery Park / rabbi conversation); February 2026 (ketamine bloom — "aged in reverse by ten years"); even early-March 2026 visit was meaningfully better. The ketamine response in particular tells us his cortical glutamatergic plasticity is preserved (per Agent 7 analysis in audits) — arguing against pure vascular depression / depression-executive-dysfunction syndrome *and* against fully-established neurodegeneration. **This is positive prognostic information for ECT planning** and worth holding onto alongside the current acute picture.

Active workup and pending consults

Consult/program	Status
U-M ECT (Dr. Nicholas Maxner)	Active path. 5/21 consultation with Dr. Maxwell (Director). Maxner callback expected early next week. Start window 2–4 weeks. Family asking U-M to inherit medication management.
U-M Movement Disorders Program	Active referral. DAT-SPECT pending. Load-bearing for the shaking question and for the broader Lewy-spectrum question (workup not yet completed; no confirmed diagnosis).
U-M Cognitive Disorders Program (Heidebrink/Paulson)	Referral planned via PCP. Parallel cognitive-workup track.
IHA Neurology	Appointment 06/16/2026 — general neurology (movement-disorder subspecialty routed to U-M).
PCP (Dr. Eric Straka, PIM Ann Arbor)	Active. 5/11 visit completed. Periodic touch.
Endocrinology	Pending — biochemical hypogonadism from 5/4 labs. Referral discussed but not yet placed.
Geriatric medicine consult	Pending referral via PCP. IHA Comprehensive Geriatric Primary Care; alternative U-M Turner Geriatric Clinic.
Consulting psychiatrist (Dr. Gabe Peltzman)	Periodic consult. Scope: medication review and second opinion.
Ketamine therapist (Julane)	Active; provides integration sessions when ketamine is used.
Ketamine clinic (A2 Ann Arbor)	Contingent. 6/8 restart was scheduled; now likely deferred given ECT direction.
Outpatient psychiatry (Dr. Rubin)	Transitioning. Currently covered by Dr. L for the holiday weekend. Long-term: psychiatric management transitioning to U-M.

Consult/program	Status
Family-physician psychiatrist (Sharon Auchincloss)	Informal family advisor. Not in care chain. Consulted by Mom on 5/23.

Recent labs and imaging

Date / test	Findings
05/04/2026 — Testosterone panel	Total T 304 ng/dL (borderline, ref 175–781); free T 3.6 (L) (ref 4.9–19.0); bioavailable T 82 (L) (ref 115–447); SHBG 70.1 (high-normal); albumin 4.2 (WNL). <i>Biochemical hypogonadism</i> . Picture: elevated SHBG sequestering testosterone such that total T appears in-range while bioactive fractions are low.
04/14/2026 — CMP	Na 141, K 4.1, Cl 105, CO2 25, BUN 21 (H), Cr 1.00, glucose 108, Ca 9.0, AST 32, ALT 24, ALP 39 (L), Tbil 0.6. eGFR 78.
04/14/2026 — CBC	WBC 6.8, Hgb 14.3, Hct 42.8, Plt 213, RBC 4.66, MCV 91.8, RDW 12.6.
04/14/2026 — Other	TSH 1.05 (WNL); B12 408; HbA1c 5.7 (pre-diabetes).
04/15/2026 — Dr. Rubin order	Lithium level, BUN/Cr, TSH, calcium. Reported normal verbally on 4/21; numerical values pending.
07/15/2025 — CT head WO contrast	"Stable" — for balance/NPH workup.
05/19/2025 — EKG	HR 50, PR 214 (borderline 1° AV block), QRS 120, QTc 401, LAFB. Asymptomatic.
PHQ-9 / GAD-7	09/09/2025: PHQ-9 21 (Severe), GAD-7 16 (Severe), SI "Not at all." 12/16/2025: PHQ-9 17 (Moderately Severe), GAD-7 7 (Mild), SI "Not at all." Current (Adam's clinical estimate): PHQ-9 ≈ 24.

Open questions being clarified

- **Distribution and pattern of the shaking** — legs only? Hands also? Rest vs. action? Pending Mom's observation 5/23.
- **Why the 11/2025 quetiapine trial failed** — relevant to current Seroquel proposal. No documented reason in available records.
- **Specifics of 11/04/2025 drug-drug interaction note (Z78.9)** — which agents.
- **Lithium level numerical value** from 4/15/2026 — verbally reported normal; written value still pending.
- **Peripheral neuropathy (G62.9, 2018)** — clinical findings and current monitoring status.
- **Pre-2024 atorvastatin → rosuvastatin switch date** — pharmacy records suggest mid-late 2024.
- **Prior-PCP records (Dr. Babe, Dr. Oberdoerster)** — pending records request.
- **December 2024 TMS course outcome** — response, durability, discontinuation reason — pending records.
- **U-M ECT acceleration response** — pending doctor-to-doctor call.
- **Dad's willingness to accept ER or voluntary admission** if recommended.
- **Whether Adult children Leah/David travel this weekend** for caregiver support.

Family situation and care coordination

- **Son Adam Plunkett** — primary point of contact for care coordination; manages clinical communication and research synthesis.
- **Wife Marcy Plunkett** — primary in-home caregiver; generally present at appointments; currently sleep-deprived and expressing "can't go through this again."
- **Daughter Leah Plunkett** — coordinates remotely; considering travel this weekend.
- **Son David Plunkett** — coordinates remotely; considering travel this weekend.
- **Sister-in-law Debbi** — arriving 5/26/2026 for one week.
- **Friend/informal advisor Sharon Auchincloss** — psychiatrist (cancer-patient focus); consulted by Mom; not in formal care chain.
- **Prior providers (relevant for records context):** Dr. Babe (interim PCP ~2024); Dr. Mark Oberdoerster (earlier PCP through ~2023); Berit (NP, considered mirtazapine in 12/2023; Dad's 1/26/2024 letter on activation-intolerance was addressed to her).
- **Family-history relevant items:** Brother Tom (living) — dementia diagnosis early 70s; longstanding alcohol use; depression. Sister Judy (deceased) — dementia around age 68; alcohol history; depression. Sister Kathy (living, no dementia). Depression and social anxiety run in family.

Allergies and critical contraindications

- **Ambien (zolpidem)** — reaction "anxiety," Active on EMR.
- **Caution with high-D2-affinity neuroleptics** — haloperidol, olanzapine, risperidone — given the pending Lewy-spectrum workup and the Lancet Neurology 2025 categorical avoidance guidance in confirmed DLB. Family asks treating providers to weigh this risk explicitly. Quetiapine at low dose (≤ 25 mg) is the conditionally-considered atypical if antipsychotic is judged necessary.
- **Activation-intolerance pattern** — caution with any new psychoactive agent; multiple prior failures across structurally different classes.
- **Cardiac caution** — sinus bradycardia (HR 49–53), borderline 1° AV block, BP 112/58. Avoid agents with strong bradycardic, hypotensive, or QT-prolonging effects.
- **Fall risk** — gait abnormality + balance problem + peripheral neuropathy + hypotension/bradycardia + CNS-depressant stack (Cymbalta + lorazepam + lemborexant $\pm$ others). Conservative ambulation and supervision in any inpatient setting.

Pharmacological glossary — agents tried, timing, reactions

Comprehensive record of every psychiatric agent and major non-psychiatric agent of relevance encountered in the past 12+ months. Status legend: **CURRENT** / **PROPOSED** / **DISCONTINUED**.

Agent	Dates	Dose	Outcome / reaction
Duloxetine (Cymbalta)	Started 09/30/2025 → CURRENT	60 mg AM	Replaced long-running escitalopram. Titrated 20 → 60 mg by 11/3/2025. Generally tolerated at 60 mg. Two short taper attempts (5/12, 5/21) at 40 mg morning dose each reversed within 24 hours due to acute anxiety. Currently held at 60 mg pending ECT.
Lorazepam (Ativan)	First dispensed 09/09/2025 → CURRENT PRN	0.5 → 1 mg; currently 1 mg PRN	Started 0.5 mg PRN 9/9/2025. Per patient self-report 4/21/2026: low PRN use (~4 doses total) since original fill. Dose increased to 1 mg by 4/2/2026. Per Rubin's 4/23/2026 plan: 1 mg daily at midday as time-limited wash-out bridge — daily use confirmed by family observation by 5/4/2026. Subsequently tapered per Rubin + Peltzman plan and reverted to PRN-only. Recent PRN doses: 1 mg 5/22 PM, 1 mg 5/23 ~8 AM. Effective for acute anxiety. Family concern: daily-use trajectory in current acute crisis.

Agent	Dates	Dose	Outcome / reaction
Melatonin	10/31/2017 → CURRENT	10 mg HS	Prescribed same day as RBD diagnosis (G47.52); for RBD specifically. Started 3 mg, later 10 mg. Continuous use 8.5 years.
Lemborexant (Dayvigo)	Tried first 5/22/2026 night	10 mg tablets prescribed; ~10 mg total taken first night	10 mg tablets prescribed (filled 5/22 CVS, \$335). First trial 5/22 night: half-tablet (5 mg) initially, then another half (5 mg) later in the night — total ~10 mg, the maximum labeled dose. Did not produce sleep. Failure could reflect the agent itself in this profile, the split-dosing pattern, or the acute sleep-deprivation/anxiety substrate overpowering the orexin blockade. Next-night question: single-dose 5 or 10 mg taken at the start of the night.
Quetiapine (Seroquel) — PROPOSED	11/03/2025 fill ~4 weeks of use; 5/17/2026 fresh 25 mg fill; proposed again 5/23/2026	Prior trial 50 mg × ~4 weeks; 5/17 fill 25 mg tabs; current proposal 25 mg HS	11/2025 trial (~4 weeks, 50 mg): discontinued because "wasn't doing anything but sedating" / "just making him sleepy" (per audits + Dec 2025 ChatGPT record). Efficacy failure at anxiety indication, not activation-intolerance/safety failure. 5/17/2026 fill (25 mg tabs): filled at CVS in response to 5/17 shaking onset (connects to OE prompt #08 proposed quetiapine 25 mg HS + 6.25–12.5 mg AM); status of whether actually taken uncertain. 5/23/2026 current proposal: Dr. L (covering) suggests 25 mg HS for sleep bridge — see proposed-meds section for full reframing of the prior-failure concern (different indication; failure mode was efficacy, not safety).
Escitalopram (Lexapro) — PROPOSED retry	Long-running through 10/05/2025; proposed retry 5/19/2026	Historical steady state 30 mg; high-dose 40–50 mg trials in 2023 produced activation/failure	Decades on this agent. Discontinued 10/2025 in favor of duloxetine. Currently proposed by Drs. Rubin + Chilton as Cymbalta replacement. Family deferring pending U-M input.
Divalproex (Depakote) — UNDER DISCUSSION	Mentioned by Chilton early-mid May; not started	125 mg BID titrate to TID proposed	Indication unclear (anxiety augmentation per Rubin; Chilton unclear if recommended or noted). Family deferring — weak evidence for VPA in unipolar TRD; DIP signal in elderly is concerning in profile under Lewy-spectrum workup; would confound Movement Disorders workup.
Doxepin (Silenor)	~05/04 → ~05/21 (~17 nights)	6 mg HS most nights; 12 mg HS one night (5/16)	Started for sleep maintenance (Rubin-prescribed). 6 mg HS from ~5/4 through 5/15 (~12 nights). Titrated to 12 mg HS one night only — 5/16. First acute shaking noted 5/17 morning — only med change that night was the 6→12 mg titration. Dr. Rubin reduced back to 6 mg HS post-5/17 shaking onset. 6 mg HS continued ~5/17 through ~5/20. Discontinued ~5/21 in transition to lemborexant. Nordoxepin (active metabolite, 31-hr half-life) accumulates over consecutive nights to steady state at ~5 days; likely meaningfully present through ~5/26.
Trazodone	Single fill 12/17/2025; PRN quarter-pill resumed May 2026 with doxepin; discontinued ~5/21	50 mg PRN HS	12/2025 single fill: tension and headache. Resumed as quarter-pill PRN in May 2026 in combination with doxepin for sleep ("okay not great" — 5.5–6 hrs sleep). Discontinued ~5/21 in transition to lemborexant.

Agent	Dates	Dose	Outcome / reaction
Olanzapine (Zyprexa)	04/09/2026 → 04/21/2026	2.5 mg → 5 mg by 4/14 PM	~12 days on board (~6 days at 5 mg). Stopped by Dr. Rubin via phone call. Reported near-panic-attack rebound on first full day off — possible activation-intolerance + withdrawal phenomenon.
Lithium	04/03/2026 → 04/21/2026	150 mg (CVS); 4/14 PCP list shows 300 mg — discrepancy unresolved	~18 days. Discontinued by Dr. Rubin same call as olanzapine. Tremors prior to discontinuation; resolved post-discontinuation. Rebound anxiety same as olanzapine.
Pramipexole (Mirapex)	03/24/2026 → ~3/26–28	0.125 mg	Discontinued within days for physical symptoms. The intolerance reaction is itself an observation worth noting (dopamine agonist intolerance is unusual but possible in patients later confirmed to have Lewy-spectrum disease; Dad's workup not yet completed).
Brexiprazole (Rexulti)	12/01/2025 → mid-March 2026	0.5 mg → 1 mg by 1/5/2026	Self-discontinued by patient — stated reason: balance issues. Patient-attributed motor symptom to medication.
Aripiprazole (Abilify)	04/02/2025 → summer 2025	2 mg	PCP note 5/19/2025 said "doing better on Abilify." Subsequent parkinsonian features summer 2025 led to discontinuation. No refills after 4/2/2025.
Mirtazapine	07/23/2025 → 10/01/2025	30 mg → 45 mg (9/4) → 30 mg (9/9)	Stopped for agitation. Activation phenotype.
Quetiapine (prior trial)	11/03/2025 single fill	50 mg	Brief failed trial. Reason for discontinuation not documented — clarification pending.
Escitalopram (Lexapro)	Decades to 10/05/2025	Historical steady-state 30 mg; high-dose 40–50 mg in 2023 → activation/failure	Long-running primary antidepressant. Switched to duloxetine 9/30/2025. Currently proposed for retry.
Sublingual ketamine	02/2026 course	10–20 mg per session, lozenge	Dramatic ~4-week response. Cognition, mood, physical activity all improved. Course completed; depression returning early March pre-procedure. Patient is documented ketamine-responder.
IV ketamine (Royal Oak)	05/18/2026 single session	Standard IV protocol	Poorly tolerated. Dissociative; shaking; "convulsive" per Mom; Dad described "going to Africa" / runaway train. No subsequent session. Family deferred A2 ketamine restart in favor of ECT direction.
TMS (standard)	12/2024 course	Standard protocol	Course completed. Response, durability, and discontinuation reason not in available records — pending TMS provider chart.
Finasteride	07/17/2025 → likely 10/2025	5 mg (BPH)	Single CVS fill. Labeled depression/SI warning; temporal proximity to F33.9 coding 5/19/2025. Likely discontinued ~10/2025 per CVS pattern. Not on 4/14/2026 PCP list.
Alfuzosin	Through 5/4/2026	10 mg (BPH)	Temporary perioperative course for prostate procedure. Confirmed discontinued 5/4/2026.

Agent	Dates	Dose	Outcome / reaction
ZZZQuil (diphenhydramine)	Briefly used early May 2026	50 mg HS	Family stopped after Adam's concern about anticholinergic burden given the Lewy-spectrum workup pending (Beers Criteria caution + PCP prior counsel against Benadryl 12/16/2025).
Oxybutynin	12/24/2024 single fill	5 mg (anticholinergic bladder Rx)	Single fill; not refilled. Likely long discontinued. Predates documented current-PCP encounters.

Related family analyses — links to recent handouts

The family has been developing topic-specific decision aids alongside this clinical brief. The following handouts reflect the family's current analytical thinking on specific medication and management questions. All are hosted at adamplunkettauthor.com/family behind a family-access gate. Listed newest-first.

- **#25 (5/22) — When is it safe for Dad to drive?** Visual driving-window timelines after Ativan, Dayvigo, doxepin+trazodone, and combinations. Quick-reference table; Depakote-doubles-everything callout; 6-step family steadiness test before driving.
- **#24 (5/21) — Sleep regimen: old stack vs. new.** Side-by-side comparison of lemborexant + melatonin vs. doxepin + trazodone + melatonin for the medium-term sleep regimen. Stacking rules and trial-window evaluation criteria.
- **#23 (5/20) — For the U-M ECT consultation.** Clinical context to share with U-M, questions to raise about ECT in Lewy-spectrum disease (Morikawa 2024 favorable safety data), GA exposure considerations, post-consultation follow-up framing.
- **#22 (5/20) — Unified medication recommendation.** [Now partially superseded by ECT direction.] Three-tier recommendation framework with comparison table; historical context for the family's medication-direction thinking pre-ECT-decision.
- **#21 (5/19) — Bupropion augmentation.** Analysis of bupropion as augmentation option in Dad's profile.
- **#19 (5/18) — Non-pharmacological sleep interventions.** NPI top-tier items: bright-light therapy, digital CBT-I, exercise, alcohol abstinence, RBD-relevant PSG. Important alongside any pharmacological sleep strategy.
- **#18 (5/17) — Quetiapine low-dose decision aid.** The contemporaneous analysis of the 5/17 shaking episode — RBD exacerbation possibility, doxepin-reduction-alone framing, cardiac stack ECG recommendation. *Directly relevant to the current Seroquel proposal.*
- **#17 (5/16) — Mirtazapine retrial.** Why Remeron was considered alongside doxepin titration.
- **#15 (5/16) — PRN anxiety options.** Alternatives to Ativan PRN for acute anxiety management.
- **#13 (5/16) — Doxepin titration decision aid.** The titration plan 6 → 12 mg (which preceded the 5/17 shaking onset).
- **#12 (5/15) — Sleep landscape.** Broad sleep-aid landscape for Dad's profile.
- **#11 (5/15) — Sleep bridge.** Bridge regimen analysis.
- **#10 (5/15) — Lemborexant decision aid.** Original analysis of lemborexant as sleep agent.
- **#08 (5/14) — Cymbalta taper timing.** The "don't run two pharmacological changes simultaneously" framing; still applies.
- **#06 (5/13) — Cymbalta → Lexapro switch.** Original analysis; family now deferring per evidence on SNRI continuation through ECT.
- **#04 (5/13) — Ketamine decision aid.** Original ketamine framing; now partially superseded by ECT direction.
- **#01 (5/13) — Depakote (divalproex) decision aid.** Original Depakote analysis; family deferring per drug-induced parkinsonism concerns in profile under Lewy-spectrum workup.

Document metadata. Sources: current-PCP (Dr. Eric Straka) EMR patient-portal export through 4/21/2026; CVS pharmacy dispensing 4/21/2023–4/21/2026; Birdi mail-order pharmacy 01/2024–04/2026; daily family observation logs; Dr. Rubin verbal communications 4/21, 4/23, 5/16, 5/19, 5/21; family-text-thread documentation 5/18–5/23; Mom's 5/23 detailed clarifications; OpenEvidence-summarized published guidance on individual clinical questions. Some events relying on family recall and verbal report from prescribers are noted with appropriate uncertainty in the open-questions section. Pharmacy dispensing dates and current-PCP EMR notes are the strongest primary sources; family-recall events have been cross-referenced across multiple family members where possible. Pre-2024 psychiatric history (escitalopram era, prior SSRI trials, possible buspirone trial Dad recalls) is incomplete pending

records from prior PCPs (Dr. Babe, Dr. Oberdoerster).

Last updated: 2026-05-23. **Supersedes:** clinical brief dated 2026-05-04. **Maintained by:** Adam Plunkett (son) on behalf of the family. **For new providers:** please direct clarifying questions to Adam in the first instance; he can route to Mom (in-home observation), Dr. Rubin (psychiatric prescribing through ~5/22), Dr. Straka (PCP), or Dr. Maxner (U-M ECT going forward).